Tepotinib monotherapy

Advanced MET exon 14 skipping lung cancer NCCP 00823a covers tepotinib on its own for adults with advanced non-small cell lung cancer carrying a confirmed MET exon 14 skipping change, after immunotherapy and/or platinum chemotherapy. It is not a first-line or adjuvant treatment guide.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Tepotinib alone · daily treatment

Active medicines: tepotinib

Emetogenic risk: Minimal to low (oral)

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Report swelling early and discuss changes in weight, movement or breathing. The team will assess the cause and agree treatment; do not start a water tablet or change tepotinib yourself.
- Agree a diarrhoea treatment plan before starting. Begin the prescribed anti-diarrhoeal medicine at the first loose stool as instructed, and maintain fluids if able. Persistent diarrhoea, weakness or reduced urine needs prompt advice.
- Tepotinib has minimal-to-low oral emetic risk. Agree an as-needed anti-sickness plan with the team, including medicine-interaction and heart-rhythm checks; no routine infusion premedication is specified.
- Ask the pharmacist to check all prescribed medicines, non-prescription medicines and supplements before changes. Tepotinib can affect medicines including dabigatran, digoxin, rosuvastatin and metformin. Supportive medicines may also affect heart rhythm; individual advice is needed.
- Avoid pregnancy during treatment. Effective contraception is needed during treatment and for at least one week after the last dose; women using hormonal contraception should also use a barrier method. Men with a partner who could become pregnant should use barrier contraception for the same period. Do not breastfeed during treatment or for at least one week afterwards. Human fertility effects are not known; discuss fertility before starting.
- The written diarrhoea prescription and escalation instructions should be confirmed before treatment. NCCP provides loperamide dosing or local-policy options; these must be individualised.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00823 v2 · p3–4 · NCCP V6 Table 5: as-needed antiemetics for minimal-low oral therapy; NCIS V8 parenteral schedules do not impose a routine oral prescription

Sources checked 2026-10-29 · SACTCheck v0.84.25 · Regimen-level drafts; prescriber and oncology-pharmacy verification required